

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)

Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$38,283,150
Prenatal arm	\$12,061,814
Postnatal arm	\$26,221,335
Administrative load (8%)	\$3,062,652
Appropriation total (benefit + admin)	\$41,345,802
Sampling 90% CI	\$29,042,602 - \$47,523,697
Assumption band	\$22,970,090 - \$48,810,481

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Statutory (Medicaid OR <=300% FPL) · 6-mo postnatal	\$4,500	\$38,283,150	\$41,345,802	16,782
Universal · 6-mo postnatal	\$4,500	\$63,793,115	\$68,896,564	27,964
Universal · 12-mo postnatal	\$7,500	\$107,487,029	\$116,085,991	27,964

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$26,221,335
Total cost (12-month design)	\$64,504,485

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

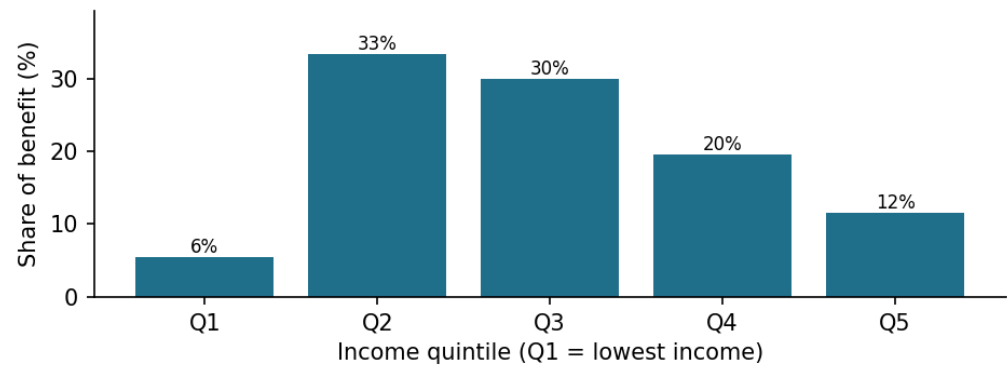
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$15,880,903
Prenatal / Postnatal	\$6,533,483 / \$9,347,420
Postnatal deferred to next FY	\$4,855,803
Year-1 as % of steady state	41%
Ramp sensitivity (6 / 12 / 18 mo)	\$24,844,715 / \$15,880,903 / \$10,587,269

Household reach

Eligible families (weighted)	7,340
Expected recipients / year	16,782
Expected pregnancies / infants	8,041 / 8,740
Avg benefit per recipient	\$2,281

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$17,182	110,267	924	\$2,108,586	5.5%
Q2	\$57,026	110,263	5,604	\$12,784,767	33.4%
Q3	\$96,722	110,317	5,030	\$11,475,307	30.0%
Q4	\$154,809	110,231	3,280	\$7,482,913	19.5%
Q5	\$334,084	110,343	1,943	\$4,431,577	11.6%

Cost by county

County	Cost total	Prenatal	Postnatal	Recipients
Hawaii	\$7,644,254	\$2,408,463	\$5,235,790	3,351
Honolulu	\$26,237,303	\$8,266,548	\$17,970,756	11,501
Kauai	\$2,601,356	\$819,605	\$1,781,751	1,140
Maui	\$1,800,237	\$567,198	\$1,233,039	789

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.